جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نشوي محمود متولي الصديق
المحافظة : القليوبيه **الحي / المدينة :** كفر شكر **العنوان :** ش الجيش
رقم البطاقة : 27807161402225 **رقم التليفون :** 01091104546 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 16-07-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نشوي محمود متولي الصديق
National ID: 27807161402225

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

DMS
تدعيم
هرمون + فيتامين د





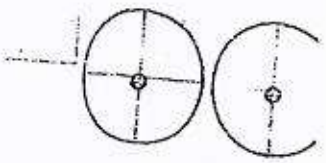
National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

0-Patient ID:
1- Name : تقوى محمود مقبل الصديق
Sex: Female
3- Medical number: 83181206
Old Med : 58-2024
2-ID number: 27807161402225
Patient Age : 46 Y



Family history	Family History	Phone	Phone 01091104546	BMI	BMI 5	REASON OF MDT	REASON OF M POST OPERAT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT 11/02/
Co morbidity	Co morbidity	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	4/8	2	Amplified	40
	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

N	M
	0

Sonomammography
lt SM inner quadrant mass 16 x15 mm + lt axillary LN

PATIENT HISTORY
lt mrm 8/2023 ILC LN 7/21 + nodal necrosis +capsular relapse , free margin
received CTH 4 AC 10/2023 till 12/2023 at elkasr el-ainy

MDT OUTCOME & PLAN

OUTCOME & PLAN
Taxotere every 3 ws/ 4 CYCLES & Herceptin SC ANADJUVANT RTH . ADJ AI AND ZOLADEX

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: نشوي محمود متولي **MR. :** 2560	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented today for the administration of Zoladex and Arimidex, with a plan to continue supportive therapy for three months.

The patient is a 48-year-old female with a history of ER 8/8, PR 4/8, HER2-positive invasive lobular carcinoma of the left breast. She underwent a left modified radical mastectomy in August 2023, but the pathology report was not available. She received postoperative radiation therapy and adjuvant AC chemotherapy at Al-Kasr before being referred to our institution for further treatment. The patient has been receiving taxane-based chemotherapy plus trastuzumab, and has continued Zoladex and Femara since then. She is scheduled for Zoladex and Femara today, with a plan to maintain this regimen for an additional three months.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **Reham Ahmed Fathy Ibrahim**

يعتمد ,

Patient Name	: الأستاذه / نشوى محمود متولى الصديق السمرى	Request No.	: 13008223
Patient No.	: 27807161402225	Request Date	: 10/01/2024
Path No.	: SR-37/24	Verification Date	: 22/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer
Nature of Specimen: Slide revision.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Moderate nuclear reaction in 30% of tumor cells (Allred score 5/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- Ki67: 40% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, Slide revision:
Invasive duct carcinoma, Grade III.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Equivocal (score 2+). For SISH.
- Ki67: Positive in 40% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / نشوى محمود متولى الصديق السمرى	Request No.	13008223
Patient No.	27807161402225	Request Date	10/01/2024
Path No.	SR-37/24	Verification Date	22/01/2024
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Age	45 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Toka Medhat

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الأستاذه / نشوى محمود متولى الصديق السمرى	Request No.	: 13008223
Patient No.	: 27807161402225	Request Date	: 10/01/2024
Path No.	: SR-37/24	Verification Date	: 22/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision.

Gross Examination:
- One paraffin block labeled by (2831102) and 12 slides were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2).
* DCIS: No.
* Skin involvement: Free skin
* Lymph nodes: Positive (5/12).

Diagnosis:
Left breast cancer , Slide revision:
Invasive duct carcinoma, Grade III.
Positive lymph nodes (5/12).
- For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy
Dr. Toka Medhat

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name	Mrs. Nashwa Mahmoud Metwally El-seceek		
Ref. By Prof. Dr.	Ahmed Hamed Rashad	Date	10/10/2023
ID	6623017478	Acc. No.	281041098

6623017478

^{18}F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of managed left breast malignancy ILC pathology 8.2023.

Total body PET scan was done 40 minutes after IV administration of 300 MBq of 18F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROI.s & the normal threshold would be the normal blood pool liver *SUV....< 2*

Reported By:

Dr. Islam Aboulwafa, MD

Approved By:

Prof. Dr.

Essam El-Shiekh

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
  (N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid (Non-homogenous thyroid gland, for further thyroid work-up) and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- *Low activity operative bed reticulations.*
- *No focal operative bed active masses.*
- *No enlarged FDG hypermetabolic mammary related lymph nodes.*
- *For mammographic correlation.*
- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- Average size of the liver with no FDG hypermetabolic focal lesions. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.
- Cholecystectomy.
- Average size of the spleen with no focal active lesions.

PET Scan
[[DOCTYPE:PET_SCAN]]

Name	Mrs. Nashwa Mahmoud Metwally El-sedeek		
Ref. By Prof. Dr.	Ahmed Hamed Rashad	Date	10/10/2023
ID	6623017478	Acc. No.	281041098

- Normal appearance of pancreas (no sizable active masses), adrenals and pelvic organs (endometrial activity with no sizable masses likely physiologic, for US correlation).
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- Faint marrow uptake of SUVmax. 2.5 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

- The operative bed changes are mostly of post management sequelae.
- The surveyed body is free from any FDG hypermetabolic neoplastic masses.
- For mammographic correlation.

Thanks for your referral;

Dr. Ahmed Kandeel
MD Nuclear Medicine

Dr. Islam Aboulwafa, MD
Prof. Dr. Essam Ali El-Shiekh

PET Scan
[[DOCTYPE:PET_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nashwa Mahmoud Metwally Al Seddik Al Semary	Date : 23/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 58/2024	Pat. ID: 98169

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Reported case of left MRM.**

I Chest:

- Status post left MRM showing no operative bed gross masses or sizable collections.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Enlarged liver showing homogeneous parenchymal attenaution. No sizable focal lesions or biliary radicles dilatation.
- Right renal cortical lesions as follows:
  1. Lower polar medial cortical hetergeonously enhancing mass lesion with central linear lucency measuring 2.8 X 2 cm.
  2. Lower polar lateral mixed solid and cystic lesion measuring 1.4 X 1.2 cm.
  3. Lower polar fat containg lesion mostly angiomyolipoma measuring 15 mm
- Normal CT appearance of the spleen, pancreas, suprarenals glands, aorta, left kidney and IVC.

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nashwa Mahmoud Metwally Al Seddik Al Semary	Date : 23/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 46 Year(s)
Medical ID: 58/2024	Pat. ID: 98169

- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- IUD is seen.
- Right iliac wing sclerotic area.
- Right femoral and left ischial sclerotic foci mostly bone islands.
- 1st degree lytic spondylolisthesis of L5

Opinion:

Status post left MRM showing no operative bed gross masses or sizable collections.

- **No sizable pulmonary or hepatic deposits.**
- **Right renal two heterogenous lesions as described, lower polar Bosniak III (lateral smaller lesion) and Bosniak IV (medial larger lesion) lesions for further assessment.**
- **Another right renal lower polar fat containing lesion mostly angiomyolipoma.**
- **Right iliac wing sclerotic area of bone scan.**

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Pat. Name: نشوى محمود متولي السمرى	Pat. ID: 662730
	Date : 20/08/2023

PELVI-ABDOMINAL SONOGRAPHY REVEALED:

*Liver: Average hepatic size with normal homogenous echopattern. No intrahepatic focal lesions or biliary radicles dilatation. Normal calibre of P.V & C.B.D.

*Spleen: Normal size & pattern.

*No ascites.

*Gall Bladder: Not visualized (cholecystectomy).

*Normal pancreas and retroperitoneal area.

*No para-aortic or mesenteric lymphadenopathy.

*Kidneys: Normal global dimensions of both kidneys with normal parenchymal thickness and echopattern. No renal calculi or back-pressure changes.

*U.Bladder: Normal appearance with no bladder calculi or intravesical filling defects.

*Normal pelvic sonography with no abnormal adnexal mass or cyst.

CONCLUSION:

☞ *Normal abdominal & pelvic U/S.*

NADA

Prof. Dr/ AHMED FARID
Dr/ MOHAMED SALAH

Sonography
[[DOCTYPE:SONOGRAPHY]]